

OREGON STATE HOSPITAL

POLICY

SECTION 6: Patient Care **POLICY: 6.045**

SUBJECT: Clinical Documentation

POINT PERSON: Health Information Director

APPROVED: Dolores Matteucci

DATE: MARCH 9, 2023

Superintendent

SELECT ONE: ☐ New policy

☐ Minor/technical revision of existing policy

☐ Reaffirmation of existing policy

☒ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. A patient's medical record is systematic documentation about a patient's condition, care, and treatment. Clinical documentation chronicles information about patients and their medical history; their responses to care, treatment, and services; and progress towards treatment goals. It is maintained for purposes of communication, accountability, and coordination of care and services.
- B. This policy establishes clinical documentation requirements, processes, and style guide information. This policy also describes draft note reports as found in the electronic health record (EHR).
- C. This policy applies to all staff.

II. POLICY

- A. At Oregon State Hospital (OSH), the medical record is contained in the designated blue medical record binder, brown medical record folders, or EHR.
- B. Staff must complete and document required assessments as indicated by applicable regulations, discipline-specific standards of practice, and department protocols.
 - 1. Staff must document clinical findings and observations about patients' response to treatment and their progress towards treatment plan goals generally on the date of service/observation. If staff cannot document on date of service, the entry must be labeled as a late entry per this policy.
 - 2. Staff are responsible for the accuracy and necessity of their documentation.

3. Documentation must be recorded in the correct medical record section.
- C. Only individuals authorized by OSH may document in the medical record.
 1. Staff must document with their own unique electronic medical record log-in. They may not share their log-in information with anyone else.
 2. Staff must clearly identify when care has been rendered by another person. They may not document an entry for another staff member.
- D. Documentation must conform to acceptable clinical documentation and medical record standards.
 1. Proper spelling, grammar, and military time must be used in handwritten entries.
 2. Symbols, abbreviations, and acronyms may be used sparingly.
 - a. If used, abbreviations and acronyms should be spelled out and/or defined the first time they are used in a document (e.g., treatment care plan [TCP]). All subsequent uses in the same document may use the abbreviation or acronym.
 - b. Only abbreviations accepted by Medical and Allied Health Professional Staff (MAHPS) may be used in the medical record.
 3. Every document filed in the paper medical record must include, at minimum, the patient's first and last name and medical record number on both front and back of the form.
 4. If clinically appropriate, the name and title of other staff may be documented in the medical record.
 5. A patient's identifying information (e.g., name, initials, or medical record number) *may not* be recorded in any other patient's medical record.
 6. Every entry in the medical record must be authenticated (i.e., finalized) with:
 - a. A legible printed name, credentials or working title, and a signature or initials. If initials are used in place of a signature, the initials of both the first and last name must be included;
 - b. Date (including month, day, and year); and
 - c. Time.
 7. Unless legitimately necessary, staff should avoid cloning (i.e., copying and pasting) information from previous documentation. If information is cloned, staff must include the original date, author, and source document.
 8. Removable notes or Post-its may not be used for documentation purposes and may not be retained in the medical record.

- E. All EHR draft status entries must be finalized within 48 hours of initiating the entry. Staff should follow Procedures A to process draft documents.
- F. Progress notes must be completed in accordance with 42 Code of Federal Regulations (CFR) 412.27(c)(4) and 42 CFR 482.61(d).
 - 1. The Doctor of Medicine or osteopathy responsible for the care of the patient, Nursing, and Social Work must document progress notes at least weekly for the first 2 months and at least one a month thereafter; and
 - 2. All disciplines significantly involved in active treatment modalities must document progress notes per frequency of services provided and as necessary as determined by the condition of the patient (i.e., as indicated per acuity of clinical presentation).
- G. Treatment Group Notes
 - 1. Group Leaders are responsible to complete group notes for each group meeting based on the group meeting schedule.
 - 2. Group notes must be documented as soon as possible, but no later than one calendar week following the group session where services are provided.
 - a. Group notes not completed on the date of service/observation must be identified as a late entry per this policy, even if still within the week deadline.
 - b. If something clinically significant occurs during a group, staff must document about the situation promptly on the date of service/observation.
- H. Late Entries
 - 1. It is expected that staff document notes on the date of service/observation. However, extenuating circumstances may prevent this from happening on occasion. Any progress note or group note not finalized on the date of service/observation must be identified as a late entry.
 - 2. Late entries must be clearly identified at the beginning of the entry and reference the actual date and time of the service provided, observation, and/or event.
- I. Editing Medical Record Entries
 - 1. In the EHR, the “append” function may only be used for its defined and intended purpose.
 - a. The original author may use the append function to correct or clarify an entry.

- b. A supervisor may use the append function to cosign, comment, or clarify information; to acknowledge a subordinate's entry; or to close documentation not finalized by the original author if they are not available.
 - c. Staff may not use the append function to comment on another staff member's entry.
- 2. Corrections to a medical record entry can be made by:
 - a. Paper - the original author may correct an entry by drawing a single line through it, initialing, and dating the correction.
 - b. Electronic – the original author should contact the OSH Health Information Department for specific guidance on correcting an entry in the EHR.
- J. In the event of EHR downtime, staff must follow business continuity processes and documentation procedures as outlined in the EHR "Downtime Binder" on each unit. Paper records entered in the EHR after the downtime should be designated as "downtime late entries."
- K. Only the Medical Records Committee may authorize an alternate clinical documentation process for good cause.
- L. Oregon State Hospital (OSH) follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- M. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. "Cloning" refers to "copying-and-pasting" blocks of text from one document to another. Cloning includes information located in pre-populated fields from earlier documents or other medical records.
- B. "Electronic health record (EHR) downtime" means any period of planned or unplanned time the EHR is unavailable.
- C. "Military time" refers to sequentially representing time using 24 hours, from 0000 [12:00 a.m.] to 2359 [11:59 p.m.].
- D. "Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

IV. PROCEDURES

Procedures A Draft Clinical Documentation Notes

V. ATTACHMENTS

VI. RELATED OSH POLICIES AND PROTOCOLS

2.009 Medical Record Forms Management

2.013 Electronic Health Record Access

2.014 Medical Record Maintenance and Transportation

2.018 Complete Medical Record

6.011 Treatment Care Planning

6.028 Learning Needs Assessment

Avatar Data Correction (ADC) form

OSH Business Continuity Processes – Avatar Downtime Binder

VII. REFERENCES

42 CFR § 412.27.

42 CFR § 482.24.

42 CFR §§ 482.60 – 482.61.

45 CFR § 164.

Centers for Medicare and Medicaid Services. (2018). Publication 100-2. Medicare Benefit Policy Manual. Chapter 2.

Centers for Medicare and Medicaid Services. (2020). *Amendments, Corrections and Delayed Entries in Medical Documentation*. Publication 100-08. Medicare Program Integrity Manual. Chapter 3.3.2.5.

Joint Commission Resources, Inc. (2023). *The joint commission comprehensive accreditation manual for hospitals*. IM.01.01.01. — IM.03.01.01. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2023). *The joint commission comprehensive accreditation manual for hospitals*. LD.04.03.01. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2023). *The joint commission comprehensive accreditation manual for hospitals*. RC.01.01.01 — RC.02.04.01. Oakbrook Terrace, IL: Author.

Oregon Administrative Rule § 333-505-0050.

Oregon Administrative Rule § 333-525-0000.

Oregon Administrative Rule §§ 851-045-0030 – 851-045-0070.

Oregon Administrative Rule § 858-010-0060.

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Oregon Administrative Rule § 877-030-0100.

Oregon Revised Statute §§ 192.553 – 192.568.